

العمر(تاريخ الميلاد) : 22-08-1972 المهنة : ربه منزل

المجلس الطبي المتخصص

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Name: هيام السيد عبادي احمد National ID: 27208220201441	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

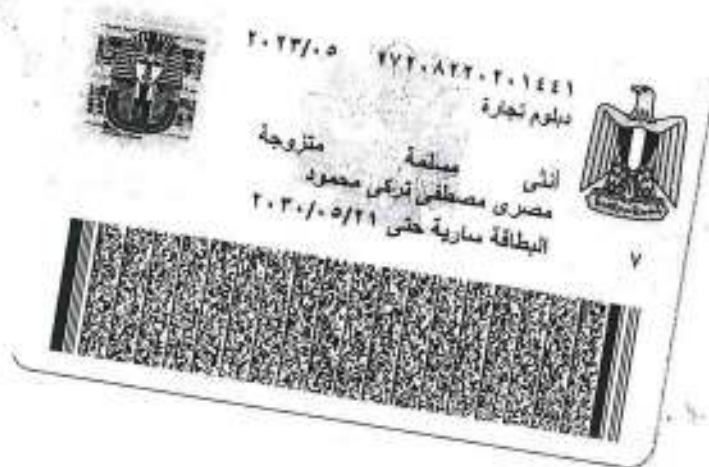
نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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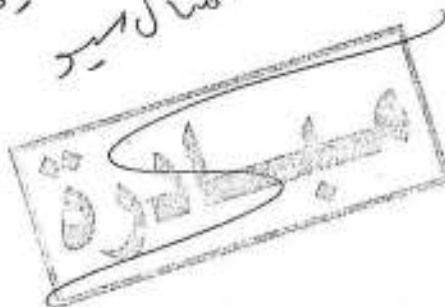
وبعد الكشف والفحوصات والمتابعة وجد أنه
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المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



فئة ملف تدوي جوي
٨ / منال سير



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مقام السيد عدنان احمد
انثى
التاريخ
24/07/2023
272082202014410
27/11/2023
HOS/NO

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Patient Name	: الأستاذة / هيام السيد عبادي احمد	Request No.	: 13003947
Patient No.	: 27208220201441	Request Date	: 09/12/2024
Path No.	: 4828/24	Verification Date	: 21/12/2024
Sex	: Female	Financial Category	: أورام دار السلام - نفقة الدولة
Age	: 52 Y	Department	: Out Patient
Referring Doc :		Registered by	: marwa mohamed youss

Pathology Report

Clinical data:

Left thyroid nodule.

Nature of specimen:

Tru cut biopsy, collected at 8/12/2024 .

Gross examination

Tint fragmented tissue cores up to 0.2 cm , totally submitted.

Microscopic examination:

Cores examined reveal variable sized thyroid follicles lined by bland cuboidal cells and filled with variable amounts of colloid.

Diagnosis:

Left thyroid nodule, tru cut biopsy:

Follicular lesion. Correlate with clinical and radiological findings.

Prof.Gamal Abdelatty

Dr.Menna Nabil

Verified by :

dr.esraa magdy





Patient Name	: الأستاذة / هيام السيد عبادي احمد	Request No.	: 13002625
Patient No.	: 27208220201441	Request Date	: 24/07/2023
Path No.	: SR-810/23	Verification Date	: 03/08/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 50 Y	Department	: Out Patient
Referring Doc :		Registered by	: Ali mostafa

Pathology Report

Clinical Data: Left breast cancer.
Nature of Specimen: Slide Revision

Gross Examination:

One paraffin block labeled by (5189/23) and three immuno-stained slides were received.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: Present
- * Microcalcifications: Positive
- * IPT:
- ER: Strong nuclear reaction in 90% of tumor cells (Allred score 8/8).
- PR: Strong nuclear reaction in 1% of tumor cells (Allred score 4/8).
- Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).

Diagnosis:

Left breast mass, Slide Revision:

Invasive duct carcinoma, Grade II, with DCIS.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 4/8).
- Her2: Positive (score 3+).

Signature
Dr. Rana Fathy
Dr. Menan Hossam

قسم الباثولوجي

Verified by :

Dr. Marwa mahmoud

Printing Date : 03/08/2023 11:06:14

Page 1 of 2

Printing By : ..marwa mohamed youss

Ministry Of Health
Specialized Medical Centers
Bab Al Salam Cancer Hospital
Radiology Department



وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
إدارة الأشعة

Patient Name: Hayam Al Sayed Abadi Ahmed	Date : 03/08/2023
Exam : C.T ABDOMEN & PELVIS	Age : 51 Year(s)
Medical ID: 2741/2023	Pat. ID: 40648

**ON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

Reported case of MBC with no available data.

1. Chest:

- Left breast UOQ irregular speculated mass lesion measuring 5.8 X 5.5 cm overlying diffuse mild skin thickening, it measures 2.2 X 1.9 cm.
- Right breast heterogenous parenchyma.
- Left axillary level I and II pathologically enlarged lymph nodes the largest measures 2 X 1.6 cm.
- Tiny middle lung lobe lateral segment nodule measuring 4 mm.
- Few (about two) bilateral scattered tiny pulmonary nodules they show internal calcifications, mostly representing post granulomatous sequel.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Enlarged heterogenous thyroid gland with internal cystic changes and calcifications for sonographic assessment.

2. Abdomen and pelvis:

- The liver is moderately enlarged with homogenous CT texture. No definite focal lesions or contour irregularities on non contrast basis.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, and both supra-renals.
- Left adnexal cystic lesion measuring 3 cm.



Patient Name: Hayam Al Sayed Abadi Ahmed	Date : 18/11/2024
Exam : ultrasound TRANS VAGINAL	Age : 52 Year(s)
Medical ID: 2741/2023	Pat. ID: 40648

Trans-vaginal ultrasound scanning revealed: -

○ **Uterus: -**

- AVF, average size uterus measuring about: 8x4x4cm in its longitudinal, transverse & AP diameters respectively.
- The myometrium shows posterior wall intra mural partially calcified fibroid measuring 20x15mm (figo4)
- The endometrial line is 6mm.

○ **Ovaries: -**

- Both ovaries are average size with no focal lesion
- No gross adnexal soft tissue masses or cysts.
- No free fluid in the Douglas pouch.

Conclusion:

Uterine fibroid

Signature Dr . Nermien Ramadan
18/11/2024 2:33 PM



Patient Name: Hayam Al Sayed Abadi Ahmed	Date : 18/11/2024
Exam : US Neck	Age : 52 Year(s)
Medical ID: 2741/2023	Pat. ID: 40648

Neck Ultrasonography

- The left thyroid lobe and isthmus of thyroid gland are enlarged in size, with average size of the right lobe
- Possible left lobe retrosternal extension
- Bilateral multiple variable size nodules, they are solid hyperechoic showing cystic areas, some show peripheral calcifications
- The largest nodule is seen at the left lobe measuring 44x26mm (TIRADS 4)
- Normal vascularity at CCDI.
- The gland moves freely with deglutition.
- The trachea is centralized.
- Both carotid sheaths are free.
- Few sub centimetric cervical lymph nodes all with oval shape and echogenic hilum likely non specific

Conclusions

Picture of multinodular goiter for clinical and laboratory correlation, the largest nodule is (TIRADS 4) for biopsy

Signature Dr . Nermien Ramadan
18/11/2024 2:31 PM

Name: Hayam Elsayed Abady
Date: 14/03/2024

51Y

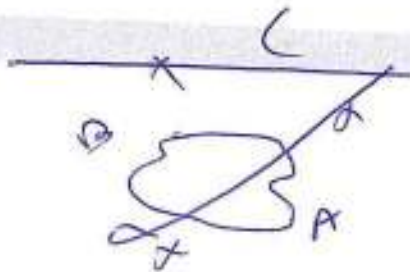
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Wire localization for left breast mass:

- Under local anesthesia, antiseptic condition and ultrasound guidance.
- Ultrasound guided wire localization was inserted into the left breast 12 o'clock mass lesion
- Whole distance of the wire till the tip: 3.8 cm
- Perpendicular distance from skin to tip: 2.5 cm
- Distance from perpendicular distance to skin entry: 3.1 cm.

Best Regards,

**Dr. Ahmed M. Rozeka, Msc
Specialist Radiologist**



A = 3.8 cm
B = 2.5 cm
C = 3.1 cm

*Dr. Ahmed
Rozeka
14/3/24*



Patient Name: Hayam Al Sayed Abadi Ahmed	Date : 17/12/2023
Exam : ultrasound BREAST (BOTH SIDE)	Age : 51 Year(s)
Medical ID: 2741/2023	Pat. ID: 40648

FOLLOW UP BILATERAL BREAST ULTRASOUND

INDICATION:

- Follow up post chemotherapy for left breast neoplastic process.
- In correlation to the previous study of 12/7/2023.

REPORT:

Regressive course as regards the size of the previously detected:

- Left breast upper areolar spiculated hypoechoic masses measuring 5.5 x 3 cm compared to 9.5 x 6.5 cm in the previous study.
- Left pathological axillary LNs are now subcentimetric.
- No supra clavicular LNs seen.
- No right breast suspicious mass lesions.
- Bilateral variable sized anechoic cysts the largest at right side measures 37 x 27 mm.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Right non specific axillary lymph nodes.

OPINION:

- Follow up post chemotherapy for a case of left breast malignancy, showing good therapeutic response, with regressive course as regards the loco-regional and nodal neoplastic burden since the last study, with residual as described (BIRADS 6).
- Bilateral fibrocystic changes.

Dr.Wafaa Ali



Patient Name: Hayam Al Sayed Abadi Ahmed	Date : 28/10/2024
Exam : MAMMOGRAPHY BOTH	Age : 52 Year(s)
Medical ID: 2741/2023	Pat. ID: 40648

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Indication:

Patient had CBS in the left breast

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- The breasts are heterogeneously dense which may obscure small lesions (ACR C)
- No right breast suspicious mass lesions, micro calcifications or architectural distortion detected.
- Right LIQ partially obscured isodense masses seen by ultrasound as cystic lesions
- Left breast UOQ focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabeculations and skin thickness.
- Complementary ultrasound showed left breast UOQ focal parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions.
- no right breast solid mass lesions, normal skin thickness and nipple-areola complex.
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free left infra and supra clavicular regions.

Opinion:

- Left breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2)
- Right fibrocystic changes (BIRADS 2)

Yours truly

Signature Dr. Walaa Abdelazez

28/10/2024 4:12 PM



Patient Name: Hayam Al Sayed Abadi Ahmed	Date : 18/11/2024
Exam : C.T.DORSAL SPINE	Age : 52 Year(s)
Medical ID: 2741/2023	Pat. ID: 40648

FOLLOWUP NON-ENHANCED MULTI-SLICE CT SCAN OF THE DORSAL SPINE REVEALED:

- Compared to the previous CT study dated 3-8-2024
- Still noted the tiny sclerotic foci seen at D4 vertebral body as well as the manubrium sterni
- Spondylodegenerative changes of the dorsal spine with marginal osteophytosis of the VEPs and variable degrees of disc spaces narrowing
- No vertebral structural collapse
- Normal appearance of the scanned apophyseal and cost spinal articulations.
- No abnormal para or intraspinal soft tissue lesions.
- Stationary course of the significantly enlarged left thyroid lobe with heterogeneous density and small calcific foci with right side tracheal shift, for US correlation and laboratory assessment

OPINION:

Stationary course since last study

Dr, Hala Sayed, MSc.

Dr. Ibrahim Abo El Magd, M.D.



Patient Name : الأستاذة / هيام السيد جيلاني احمد
Patient No. : 27208220201441
Path No. : 1048/24
Sex : Female
Age : 51 Y
Referring Doc :

Request No. : 13004613
Request Date : 16/03/2024
Verification Date : 24/03/2024
Financial Category : أورام دار السلام - نفقة الدولة
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

- Angioinvasion: Not detected.
- Ductal carcinoma in situ: Focal (5%).
- Lymph node involvement: Free (0/21).
- Resection Margins:
 - Extra deep margin showed atypical ductal hyperplasia.
 - Free other resected margins.
- Histological response to therapy: Pathologic partial response.

Diagnosis:

Left breast cancer post NACT, wide local excision and axillary clearance specimens:

- Invasive duct carcinoma, grade II.
- Tumor cellularity: 50%.

Resected margins:

- Extra deep margin showed atypical ductal hyperplasia.
- Free other resected margins.
- Free lymph nodes : (0/21).
- Pathologic partial response.
- Stage : ypT2, ypN0.
- RCB: 2.099, class: II.

Comment: There is a disparity between frozen and paraffin results, however final diagnosis is based on paraffin results.

Prof. Gamal Abdelatty
Dr. Esraa Magdy

Verified by :
Dr. Heba Hasan

Printing By : marwa mohamed

Printing Date : 28/03/2024 10:33:40

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